

SOUTH CAROLINA DEPARTMENT OF HEALTH AND HUMAN SERVICES

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PALMETTO COORDINATED SYSTEM OF CARE SERVICES PROVIDER MANUAL

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South Carolina Department of Health and Human Services

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PROGRAM OVERVIEW

PALMETTO COORDINATED SYSTEM OF CARE

The State of South Carolina is developing the Palmetto System of Care (PCSC) for South Carolina's children and youth with significant behavioral health challenges or co-occurring conditions in or at imminent risk of out-of-home placement. PCSC is an evidenced-based approach that is part of a national movement to develop family-driven and youth-guided care, and keep children at home, in school, and out of the child welfare and juvenile justice systems. The PCSC goal is for children and families to receive services when needed and as indicated by medical necessity, to support safe and healthy functioning.

The purpose of this PCSC Waiver Manual is to outline services available under the Waiver, to include home and community-based supports and services to children with mental illness who would otherwise be served in inpatient general and psychiatric hospitals. Families and youth are offered the choice of behavioral health services and supports to permit the youth to remain in, or return to, the least restrictive environment preferably their homes.

WAIVER DESCRIPTION

PCSC services are provided using a system of care approach. There are four regional points of entry across the state for all waiver participants. Applicants are evaluated regionally to determine eligibility for the waiver. Families and youth who enter the waiver participate in person-centered plan development meetings made up of individuals involved with the family and any representatives the family chooses to attend. During the meeting, a person-centered plan is developed. The family may choose from enrolled qualified providers that are composed of both public and private providers. If the family has a provider who they have been working with who is not a qualified Medicaid provider, efforts are made to contact the provider to discuss their interest in enrolling. The child and family team also known as the service plan development team, meets at least every 90 days to discuss treatment progress and any changes that are requested or more often as needed or requested. Annual reevaluations are conducted to determine continued eligibility for waiver participation.

ADMINISTRATION

The PCSC Waiver is operated by South Carolina Continuum of Care (COC) and administered by SCDHHS, the state Medicaid Agency. SCDHHS will provide guidance and oversight to participating agencies. This ensures compliance with waiver requirements and other rules and regulations under the provisions of the approved 1915(c) and of the approved 1915(b)(4) home- and community-based waivers.

COC, a provider of High Fidelity Wrap-around services affords waiver participants and their families intensive care coordination, advocacy and support. Intake functions for the waiver are conducted by COC staff. COC uses the Child and Adolescent Service Intensity Instrument (CASII) screening tool to assess the prospective participant's intensity of need and eligibility for waiver services. If it is determined that a prospective youth is not eligible for the waiver services, COC will refer the individual to other appropriate services that can meet their needs. In order to qualify for waiver services, the child must meet Medicaid financial eligibility, meet the age requirements for the waiver and meet the waiver level of care requirement and high-fidelity wraparound eligibility requirements.

If the CASII assessment indicates that the participant needs waiver services, COC will discuss the service options with the family. Families who opt for waiver services can begin receiving services once they have signed the Rights and Responsibilities form and the Grievance and Appeals form. Samples of the form can be found in the Forms sections of this manual and can be accessed via Phoenix. Families seeking waiver services will be assigned a Wrap Facilitator before they are eligible to begin receiving services.

ASSURANCES

SCDHHS and COC must guarantee that waiver services are inclusive of the HCBS assurances.

Waiver Administration and Operations

As the administrative authority, SCDHHS exercises oversight of the waiver program by monitoring the performance of waiver functions performed by other state agencies and contracted entities. SCDHHS demonstrates adherence to approved rules, policies and procedures governing the waiver program by COC and other contracted entities through documenting:

- Number of waiver policy changes approved by SCDHHS prior to implementation

Evaluation of Need/Level of Care

COC Providers assure an initial evaluation (and periodic reevaluations, at least annually) of the need for a level of care specified for this waiver, when there is a reasonable indication that an individual might need such services in the near future (one month or less) but for the receipt of home and community-based services under this waiver. The COC demonstrates implementation of approved processes and instruments for evaluating and re-evaluation level of care through documenting:

- Number and percent of youth with initial level of care determinations reviewed that were completed using the process required by the approved waiver
- Number and percent of applicants referred for LOC assessment with a brief screen indicating applicant could meet inpatient hospital LOC.

Choice of Alternatives

COC assures that when an individual is determined to be likely to require the level of care specified for this waiver and is in a target group the individual (or, legal representative, if applicable) is:

Informed of any feasible alternatives under the waiver; and, given the choice of either institutional or home and community-based waiver services.

Waiver Enrollment

When capacity is available, entry to the waiver is offered to individuals on the date of their application for the waiver (first come first served basis). Eligible applicants will be placed on a waiting list when capacity is not available. Reserve capacity when available, allows priority admission to the waiver for the purpose of diverting youth from institutionalization or serving youth leaving institutions. Reserve capacity or “crisis slots” are designated in the Phoenix system.

COC assures the selection of applicants are enrolled in the waiver program in accordance with the state’s policies by ensuring applicants are enrolled according to date of eligibility.

HCBS Settings Compliance

SCDHHS and COC assure waiver services are provided in HCBS-compliant settings by documenting:

- Number and percent of participants residing in traditional households meeting home and community-based settings at enrollment and annually.
- Number and percent of participants residing in non-traditional settings that meet DSS licensing standards as outlined in the approved waiver at enrollment and annually.

Service Plan Development

All waiver participant service plans must meet applicable standards. Enrolled waiver Medicaid providers, including COC and respite providers, are responsible for ensuring compliance with the following assurances during the service plan development process and throughout service delivery:

- Number and percent of participants whose person-centered plans address their needs, and personal goals identified during the assessment process.
- Number and percent of service plans that involved participants and/or responsible parties in the development process during the waiver year.
- Number and percent of person-centered plans updated every six months.
- Number and percent of person-centered plans updated as participants needs change.
- Number and percent of participants who received services and, in the amount, type, frequency and duration as identified in the person-centered plan.

Qualified Providers

Waiver services must be provided by qualified providers. SCDHHS and COC assures licensed and non-licensed providers meet and adhere to required standards and are trained in accordance with the approved waiver. The following must be documented:

- Number and percent of new HCBS providers that meet initial and ongoing licensure and/or certification enrollment requirements.
- Number and percent of enrolled licensed and non-licensed HCBS providers completing annual background/registry check waiver requirements.
- Number and percent of HCBS providers whose staff meet training requirements.

Health, Safety and Welfare

Providers assure that necessary safeguards have been taken to protect the health and welfare of persons receiving services under this waiver. These safeguards include:

- Assurance that the standards of any state licensure or certification requirements specified in **Appendix C** (which details provider standards for credentialing) are met for services or for individuals furnishing services that are provided under the waiver. The state assures that these requirements are met on the date that the services are furnished; and,
- Assurance that all facilities subject to §1616(e) of the Act where home and community-based waiver services are provided comply with the applicable state standards for board and care facilities as specified in **Appendix C** (which details licensing of facilities).

SCDHHS and COC further monitors participant health and welfare through documenting:

- Number and percent of new participants or guardians acknowledging receipt of information on how to report abuse, neglect, exploitation and other reportable incidents.
- Number and percent of suspected cases of abuse, neglect and exploitation where recommended actions to protect health and welfare were implemented.
- Number and percent of critical incidents reported in required time frame as specified in state policy, aggregated by incident type-abuse, neglect, exploitation, or misappropriation of funds.
- Number and percent of critical incidents that were monitored by the Critical Incident Specialist (CIS) until appropriate resolution.
- Number and percent of participants with a critical incident (CI) with a prevention plan due to abuse, neglect, exploitation, (ANE) or misappropriation of funds.

- Number and percent of participant deaths with a determined need for investigation that were investigated.
- Number and percent of participants with an identified need for medication administration during the waiver year.
- Number and percent of instances of restraint, seclusion or other restrictive intervention with a prevention plan developed as a result of the incident.
- Number and percent of participants in need of medication administration with a resulting medication administration plan during the waiver year.

Financial Accountability

SCDHHS must demonstrate an adequate system has been developed to ensure financial accountability of the waiver program. The following must be documented:

- Number and percent of claims for authorized waiver services submitted with the correct service code.
- Number and percent of HCBS claims verified through the claims compliance audit to have paid in accordance with the participant's waiver plan of care during the waiver year.
- Percent of claims paid for participants who were eligible for services and when the services were provided by a qualified provider during the waiver year.
- Number and percent of providers who have payment recouped for waiver services due to a lack of proper supporting documentation during the waiver year.
- Number and percent of waiver claims submitted with the correct rate as specified in the waiver application.

SCDHHS and COC monitor the waiver assurances via the Phoenix system to guarantee they are being met. Providers who are not in compliance may not receive reimbursement for services.

FREEDOM OF CHOICE

Freedom of choice, for waiver participants, means that an individual can choose to receive home and community-based services, rather than receiving services in an inpatient hospital or other out-of-home placement. Waiver participants can also choose to make informed decisions regarding staff and provider changes, when they are no longer satisfied with the supports and services they are receiving.

Eligible Medicaid beneficiaries who are enrolled in the waiver services, and who meet the level of care for the waiver, have the right to choose regarding the environment in which services are delivered, as well as a choice of who delivers the services. Eligible Medicaid beneficiaries, who will be enrolled in the PCSC Waiver, are informed about South Carolina's service options. This allows

them to make an informed decision about how they want their services delivered. Waiver participants have the right to discontinue waiver services at any time, and for any reason. There are no restrictions on how often a family can change providers. When a family requests a change in a waiver service provider, the request will be processed by COC. At the time of the request, the COC will be available to discuss the repercussions of changing providers; for example, the impact that a provider change might have on the treatment that the participant receives. Families will never be discouraged from changing providers; however, efforts will be made to resolve any issues or concerns between the family and the current provider.

If a family determines that they would like to discharge from the waiver, or change providers, they should communicate their preference to their COC Wrap Facilitator. If the COC Wrap Facilitator fails to complete the task or is not otherwise available, the family can communicate with SCDHHS.

The 1915(b)(4) waiver allows South Carolina to selectively contract with the South Carolina Continuum of Care for High Fidelity Wraparound services approved under the concurrent 1915(c) HCBS waiver and provides a waiver of section 1902(a)(23) related to freedom of choice. As a result, waiver participants will receive High Fidelity Wraparound services from the South Carolina Continuum of Care.

When identified on the person-centered plan of care, waiver participants will be able to choose from among qualified respite providers.

A sample of the Freedom of Choice form can be accessed via Phoenix.

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COVERED POPULATIONS

ELIGIBILITY/SPECIAL POPULATIONS

Eligibility for Services

Targeting and Clinical Criteria

COC staff will determine that the applicant meets the clinical and targeting criteria for the waiver.

If the individual meets the following criteria, they may receive PCSC Waiver services.

Medicaid-eligible individuals must meet the following clinical eligibility to be considered:

- Age: A youth must be up to 21 years old.
- Diagnosis: a primary mental health diagnosis meeting the targeting criteria
- Functional Assessment: All youth on the waiver must meet minimum scores for the hospital level of care as determined by the Child and Adolescent Service Intensity Instrument (CASII).

A clinical redetermination occurs annually.

Financial Eligibility

If a youth is not already eligible for Medicaid, a financial eligibility determination for Medicaid is completed by SCDHHS following the level of care determination. A financial redetermination occurs annually.

Applicants who meet all eligibility requirements will be approved by the SCDHHS for enrollment and participation in PCSC Waiver services.

Grievances and Appeals

Grievance

A grievance is an official complaint and may include dissatisfaction with a provider, a service or how the waiver functions. SCDHHS is responsible for operating the grievance system for the PCSC waiver. All complaints must be submitted in writing. If the written statement is received by the Wrap Facilitator a supervisor will investigate and schedule a meeting to address the complaint. When the written statement is received by SCDHHS, waiver staff will investigate and schedule a meeting to address the concerns.

When the complaint is resolved during a meeting, a written statement of resolution is documented in the participant record. Unsatisfactory decisions may be appealed by writing to the SCDHHS Waiver Director. SCDHHS must issue a written decision within 10 days of receiving the complaint.

Appeals

The purpose of an appeal is to provide a person an opportunity to question an adverse action or determination of a final agency decision. Examples of adverse actions include termination, suspension or reduction of waiver or Medicaid state plan services on participant's plan of care. Examples of adverse determinations include denial of entry into the waiver. A final agency decision is the completion of the decision-making process.

Appeals can be requested if:

- An applicant is not given the choice of home and community-based services as an alternative to institutional care;
- A participant is denied a service or provider of choice (when available); or
- A participant has services denied, suspended, reduced or terminated.

To expedite the process, appeals should include the following:

- A copy of the notification of denial received from SCDHHS regarding the specific matter on appeal;
- A specific statement identifying which issue is being appealed; and
- A description of how the family member or applicant/participant will receive status updates on the appeal request.

To avoid interruption of waiver services, an appeal must be submitted within 10 calendar days of the date the written notification of the adverse decision or determination was mailed. If the appeal is denied, the family member or participant may be required to pay for services during the appeal period. Otherwise, an appeal must be submitted within 30 calendar days from the date the notification was mailed.

Appeals can be submitted in three ways.

- Online at:

<https://msp.scdhhs.gov/appeals/site-page/file-appeal>

- Mailed to:

Division of Appeals and Hearings
SC Department of Health and Human Services
PO Box 8206
Columbia, SC 29202-8206

- Faxed to:

(803)-255-8206

Concerns and complaints regarding services being provided to children by a state agency can be submitted to South Carolina Department of Children's Advocacy (DCA) at:

<https://childadvocate.sc.gov/submit-complaint>

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ELIGIBLE PROVIDERS

GENERAL PROVIDER QUALIFICATIONS

Enrollment

Providers must comply with provider enrollment policies to continue as qualified waiver providers. The provider enrollment policy requires revalidation for Medicaid providers. In addition, all current providers must meet the provider enrollment standards at the time of revalidation. Each provider must:

- Identify their interest in being a PCSC Waiver provider
- Meet the standard provider enrollment requirements
- Meet the supplemental PCSC Waiver enrollment requirements
- Complete enrollment as a PCSC Waiver provider

Organizations who successfully meet all enrollment criteria established by the Medicaid program, and who are designated as qualified providers, must determine which of the following PCSC Waiver services they can deliver. The following services, which are provided under the waiver, must be in accordance with this policy manual:

- High Fidelity Wraparound
- Respite care

Enrollment in the South Carolina Medicaid program does not provide a guarantee of referrals or a certain funding level. Additionally, enrollment in SC Medicaid does not guarantee selection as a PCSC Waiver provider. Failure to comply with all Medicaid policy requirements may result in termination of Medicaid enrollment.

As a condition of Medicaid program participation, providers must ensure that adequate and correct fiscal and medical records are kept. Such disclosures will indicate the extent of services rendered and will also ensure that claims for funds are in accordance with all applicable laws, regulations, and policies.

All delivered services and claim submissions shall be in compliance with applicable federal and state laws and regulations, SCDHHS policies and procedures, and Medicaid provider manuals.

Reporting Changes

Changes affecting business operations must be reported, in writing, to the Medicaid Provider Service Center, as soon as possible. Certain changes may impact your status as a Medicaid provider. The following changes must be reported, on company letterhead, by the director/Chief Executive Officer (CEO):

- Physical addresses, e-mail addresses, or telephone numbers for the business office
- Change of location, or adding a location
- Director or CEO
- Clinical Director
- Staff licensure
- Business licenses
- Accreditation status
- Change in ownership
- South Carolina Department of Health and Environmental Control (SCDHEC) Residential Facility licensure
- DSS licensure (as applicable, e.g., Foster Care Respite providers)
- New hires
- Other changes, which affects compliance with Medicaid requirements

Exceptional circumstances may require that a new Enrollment Application for the PCSC Waiver be completed prior to approval.

Closure of a PCSC Waiver Provider

In the event the provider is no longer operational and closes for business, the provider will adhere to all applicable federal and state laws, rules, and regulations, including but not limited to, the following requirements:

1. If the provider voluntarily terminates his or her agreement with Medicaid, a written notification must be received by SCDHHS and other appropriate agencies within 30-days of closing the facility. The notification shall include the location where the beneficiary and administrative records will be stored.
2. If the provider is terminated, involuntarily, by Medicaid, the provider is responsible for all beneficiary and administrative records, in the event of a post-payment review.

The owner(s) of the PCSC Waiver business entity provider is responsible for retaining administrative and beneficiary records for three years.

Quality Improvement

PCSC Waiver providers must self-monitor their adherence to waiver policies, rules, regulations, and laws, to ensure they are in compliance. At a minimum, SCDHHS will conduct annual administrative reviews as needed to ensure that providers are in compliance with Medicaid policies, state and federal laws, and requirements for the waiver. Other authoritative entities will conduct quality reviews of PCSC Waiver providers, as needed.

All PCSC Waiver providers must maintain and make available upon request, appropriate records and documentation of such requirements, qualifications, trainings, and investigations. Providers are given five business days to retrieve the records for those authoritative agencies who have requested the documents. All providers of PCSC Waiver services shall maintain a file that ensures that each staff member meets the qualifications of the job that they perform. In addition, the service providers must submit a list of all staff (containing names, signatures, and initials) that administer waiver services. Additionally, copies of all staff credentials and degrees conferred are necessary.

Any services that are provided by staff who do not meet SCDHHS staff qualification requirements are subject to recoupment.

SERVICE-SPECIFIC QUALIFICATIONS

Specific staff requirements are detailed below. However, all providers must be on the Medicaid Qualified Provider List.

High Fidelity Wraparound Staff

SCDHHS will contract solely with the COC to perform the HFW service. Originally established in 1983 by the South Carolina General Assembly, the COC for Emotionally Disturbed Children now the COC, is a South Carolina state program that serves children with serious emotional or behavioral health diagnoses whose families need help keeping them in their home, school or community. The COC implemented the wraparound model in 2014 and is the only provider of HFW in the state of South Carolina.

COC must ensure on-going training of all child and family team members either through contract with national bodies or by employing nationally certified trainers and must ensure that all child and family team members meet all conflict of interest requirements in the HCBS regulation.

More specifically, the CMS rule (42 CFR § 441.301(c)(1)) requires that HCBS programs use a person-centered planning process which includes ways to solve conflict or disagreement and that the guidelines around conflict of interest are clear to everyone involved in the planning process. The rule also requires that providers of HCBS, or those who have an interest in or are employed by a provider of HCBS, may not provide case management to or develop the person-centered service plan for people receiving services.

Individuals enrolled in the PCSC Waiver must be provided with a clear and accessible way to solve any issues.

The wraparound facilitator, wraparound team lead, or wraparound coach/supervisor must be credentialed or in the process of being credentialed by a national accrediting body as meeting the standards of High Fidelity Wraparound and demonstrate continued use of evidenced-based wraparound standards as verified by SCDHHS through COC's ongoing participation in wraparound fidelity monitoring.

The wraparound facilitator must meet the following:

- A bachelor's degree in a human service or social service-related field.
- One year of experience with children with serious emotional or behavioral health challenges.
- Completion of the required training for evidenced-based, high fidelity wraparound process for wraparound facilitators.
- Pass a South Carolina criminal history background check, child abuse and neglect screen, motor vehicle screens and excluded provider screen.

The wraparound team lead must meet the following minimum requirements:

- Must have professional experience in human services or social services program.
- A bachelor's degree in a human service or social sciences related field.
- At least 3 years of experience with children with serious emotional and behavioral health challenges.
- Experience in provision of High Fidelity Wraparound and maintain agency standards as a regional mentor.
- Completion of the required training for evidence-based, High Fidelity Wraparound process for wraparound facilitators.
- Pass a South Carolina criminal history background check, child abuse and neglect screen, motor vehicle screens, and excluded provider screen.

The wraparound coach/supervisor must meet the following minimum requirements:

- Professional experience in human services or social services programs, a master's degree in a human service or social sciences related field.
- Must have 2 years of case management experience and experience with children with complex emotional or behavioral health challenges.

- Requires a minimum of one year of supervisory experience. Must be licensed and in good standing as a LMSW, LPC, LISW-CP, or LMFT in South Carolina.
- Obtain certification as a State Wraparound coach within an established time frame and remain in good standing with National Wraparound requirements and the South Carolina Department of Labor Licensing and Regulation.
- If the coach/supervisor also functions, in part, as a wraparound facilitator, they must also meet the requirements for a wraparound facilitator described above.
- The wraparound facilitator supervisor must provide regular supervision to wraparound facilitation service delivery staff, including completion of all supervisor requirements for wraparound fidelity monitoring.
- The wraparound facilitator supervisor must have good interpersonal skills for supporting development in others. The supervisor must collaborate closely with other supervisors in other child-serving agencies in the community.
- A wraparound supervisor should demonstrate skills that support engaging people from different cultures, ages and backgrounds. A preferred supervisor characteristic is an understanding of, and experience with, different systems, including schools, behavioral health, child welfare, juvenile justice, health and others. The wraparound facilitator supervisor must oversee the work of the wraparound facilitation service delivery staff on an ongoing basis.

All Wraparound services will be provided via COC and certification is required.

Respite Agencies/Providers

Respite care includes services provided on a short-term, planned or emergency basis, and offers relief to a beneficiary's unpaid caregiver who is unable to provide services to the participant. This service will be provided to meet the participant needs, as per the person-centered plan. Beneficiaries are encouraged to receive respite in the most integrated and cost-effective settings.

Respite may not be provided by any person who is a legal guardian or acting in the place of a legal guardian for the youth. A person with informal responsibility for the care of the youth such as a neighbor, friend, or extended family member may register with any of the provider types listed below. They must meet the provider's qualifications for employment, become an employee of the provider, and provide respite services for the youth. Respite may not be provided by any spouse or domestic partner of any person who is a legal guardian or acting in the place of a legal guardian for the youth.

Home Health Agencies

The Home Health Agency must maintain certification and licensure as required by the state in which it is located. The agency must also:

- Comply with SCDHHS standards, including regulations, policies and procedures relating to provider qualifications and enrollment.
- Complete and ensure employees complete SCDHHS required training, including training on the youth's person-centered plan and the youth's unique and/or disability specific needs, which may include, but is not limited to, communication, mobility, and behavioral needs.

Individuals employed by the agency must:

- Be at least 18 years of age.
- Pass a South Carolina criminal history background check, child abuse and neglect screen, motor vehicle screen, and excluded provider screen.
- In the case of direct care personnel, possess certification through successful completion of training program as required by the SCDHHS.

Supportive Housing Agencies

The Supportive Housing Agency must maintain certification as required by the state in which it is located. The agency must maintain staffing necessary for the health and welfare of the transition age youth. The agency must also:

- Comply with SCDHHS standards, including regulations, policies and procedures relating to provider qualifications and enrollment.
- Complete and ensure employees complete SCDHHS required training, including training on the youth's person-centered plan and the youth's unique and/or disability specific needs, which may include, but is not limited to, communication, mobility, and behavioral needs.
- Document three years of experience in providing services to persons with SED or SPMI.
- Comply with and meet all standards as applied through each phase of the standard, annual SCDHHS performed monitoring process.
- Ensure 24-hour access to personnel (via direct employees or a contract) for response to emergency situations that are related to the residential supports or other waiver services.

Employees of the agency must:

- Be at least 18 years old and have a high school diploma or equivalent.
- Pass a South Carolina criminal history background check, child abuse and neglect screen, motor vehicle screen, and excluded provider screen

- In the case of direct care personnel, possess certification through successful completion of training program as required by the SCDHHS. If providing nursing care, must have qualifications required under State Nurse Practice Act (i.e., RN or LPN).
- Have a valid driver's license if the operation of a vehicle is necessary to provide the service.

All supervised housing staff must complete the following training:

- CPR
- First Aid
- Introduction to Community-Based Residential Services for Direct Care Staff
- Air and Blood Borne Pathogens
- Non-Physical Crisis De-Escalation, Crisis Management and Debriefing
- Proper Techniques to address Challenging Behavior and Proper Contingency Management
- Rights and Responsibilities of Individuals receiving Mental Health services
- Cultural Competence and Diversity
- Prevention/Intervention and Recovery/Resiliency Strategies
- Behavioral Health/SUDs and Associated Medical Care and Conditions
- HIPAA and Confidentiality
- Applied Suicide Intervention Skills
- Environmental Emergencies: Mitigation, Preparation, and Responding
- Basic Health and Medications
- Assessing mobile crisis need

Public or Private Child Service Entities

Public or private child services entities must maintain certification from the South Carolina Department of Social Services (SCDSS). Entities must also:

- Comply with SCDHHS standards, including regulations, policies and procedures relating to provider qualifications and enrollment

- Complete and ensure employees complete SCDHHS required training, including training on the youth's person-centered plan and the youth's unique and/or specific needs based on ability status, which may include, but is not limited to, communication, mobility, and behavioral needs
- Be in good standing per State Medicaid regulations. The foster home parents and agency staff must be licensed and/or credentialed per State Medicaid policy.
- Ensure that all employees participate in required annual training per waiver and State Medicaid policy

Individuals employed by the entity must:

- Be at least 21 years of age.
- Pass a South Carolina criminal history background check, child abuse and neglect screen, motor vehicle screen, and excluded provider screen.
- In the case of direct care personnel, possess certification through successful completion of training program as required by the SCDHHS.

Supervisors of Respite staff who provide services in the youth's home must ensure that all employees participate in all required training. Supervisors of Respite staff must meet one of the following requirements:

- A registered nurse licensed to practice in South Carolina and have at least three years of experience performing clinical or case work activities or
- Have met one of the supervisor criteria listed below for in-home services
- Adhere to the guidelines as stipulated by respective credentialing bodies/boards

Respite provided in the youth's home

An individual who provides respite services in the participant's home must be employed through an enrolled respite provider and successfully complete all required training. They must meet the following requirements:

- Be at least twenty-one years of age or older;
- Have knowledge of the needs of children and be capable of meeting the needs of youth in the waiver;
- Be capable of handling an emergency situation;
- Respite staff must take a minimum of fourteen (14) hours of appropriate Respite care training and expected standards of care;

- A minimum of three written letters of reference shall be initially obtained prior to the Respite worker providing respite services to a youth in the youth's home. References should have known the applicants three years prior to the application and, unless specifically requested, should not be related to the applicants.

SUD Residential Facilities

SUD residential facilities must be licensed by SCDHEC. They must also hold a certification by DAODAS as either a SUD residential or detoxification program and be authorized to provide a specified number of licensed beds.

Non-Institutions of Mental Disease (IMD) Group Homes

Non-IMD group homes must maintain a staffing ratio necessary to keep youth safe, as stipulated by SCDHEC. An adult staff member must be awake and present at all times in the facility. If cameras are used, the staff must monitor the cameras and be within walking distance of the youth.

Group homes must have the availability of a clinician who is on call 24/7 for emergencies and staff consultation. The clinician must hold a doctoral or master's degree in clinical or counseling psychology, mental health nursing, clinical social work, vocational/psychiatric rehabilitation or education from an accredited college or university; or a registered nurse with certification in mental health nursing from the American Nurses Association. The home will also have access to a physician after hours for emergencies.

Group homes must employ a Residence Manager. The Residence Manager is responsible for the operation of the group home and responsible for the supervision of residents' treatment plans. The qualifications of the Residence Manager must be at least a Bachelor's degree.

Group homes also employ Residential Service Assistants who are individuals with a high school diploma, GED, or CNA.

The service provider shall maintain a current personnel policies and procedures manual that sets forth grounds for termination, adequately supports sound resident care and is made readily available to the program's staff in each home. The service provider shall comply with the provisions of such manual.

- Training in risk assessment of dangerousness and interventions aimed at reducing such risk, including training in managing difficult behaviors, in the implementation of de-escalation techniques, and in self-defense techniques to prevent harm from violent behaviors Note: Training in the use of alternatives to seclusion and restraint is required
- A complete course in medications used in the treatment of mental illness including the medications' effects, side effects, and adverse effects (sometimes life threatening) used alone or in combination with other prescription and non-prescription medication and alcoholic or caffeinated beverages

- Training in the common types of mental illness including signs and symptoms of schizophrenia, mood and personality disorders and indications of deterioration of an individual's mental condition
- Training in basic first aid, including basic CPR training and fire safety and evacuation procedures
- An explanation of the rights of youth with psychiatric disabilities in residential care in South Carolina
- Expectations for confidentiality and ethical behavior towards residents who will reside in the group home;
- Policies and procedures that apply to a group home on both a daily and emergency basis
- Health care, sanitation, and safe handling of food
- Orientation to situational counseling, de-escalation and mediation techniques, stress management and social interaction, and
- A plan for the continuing education and development of staff including but not limited to: A protocol and training for all direct care staff on the medication protocols in place for the home (i.e., a nurse or pharmacy sets up the medication and the medications are in daily packages in a locked cabinet. The staff's role is to make sure the right medications are taken at the right time by observing the youth, but not to "administer" them. A nurse verifies that the medications are taken daily.)
- Training for all direct care staff on any side effects of medications given to house residents, to promote monitoring for symptoms that reflect the need for some intervention.
- Training on basic physical health and symptoms to monitor for when a youth is in a residential program.

All service providers shall comply with criminal background check and drug testing laws.

Rehabilitative Behavioral Health Services (RBHS) Providers

RBHS providers must be enrolled with SCDHHS as RBHS providers. Additionally, they must meet requirements for any licensure/certification and appropriate standards of conduct by means of evaluation, education, examination, and disciplinary action regarding the laws and standards of their profession as promulgated by the South Carolina Code of Laws and established and enforced by the South Carolina Department of Labor Licensing and Regulation.

The presence of licensure/certification means the established licensing board in accordance with SC Code of Laws has granted the authorization to practice in the state. Licensed professionals must

maintain a current license and/or certification from the appropriate authority to practice in the State of South Carolina and must be operating within their scope of practice.

The following professionals possessing the required education and experience are considered clinical professionals/paraprofessionals and may provide Medicaid RBHS in accordance with South Carolina State Law as long as they are enrolled with the SCDHHS as RBHS providers:

- Licensed Psychologist
- Licensed Independent Social Worker, Clinical Practice
- Licensed Professional Counselor
- Licensed Master of Social Work
- Licensed Addiction Counselor
- Licensed Bachelor of Social Work
- Registered Nurse
- Licensed Practical Nurse
- Substance Abuse Specialist
- Mental Health Specialist
- Child Service Professional
- Behavior Analyst (Bachelor's level)

Rehabilitative behavioral health services provided by licensed/certified professionals must follow supervision requirements as required by SC State Law for each respective profession.

Rehabilitative behavioral health services provided by any unlicensed/uncertified professional must be supervised by a master's level clinical professional or licensed practitioner of the healing arts (LPHA). Substance Abuse Professionals who are in the process of becoming credentialed must be supervised by a Certified Substance Abuse Professional or LPHA.

The following licensed professionals are considered a LPHA: psychiatrist, physician, psychologist, physician's assistant, advanced practice registered nurse, registered nurse with a Master's degree in psychiatric nursing, licensed independent social worker -clinical practice, licensed master social worker, licensed marriage and family therapist and licensed professional counselor.

NOTE: All respite provider types re-verify annually unless a critical incident occurs, at which time a re-verification is required, as specified by SCDHHS.

Respite service provided in a certified residential care facility, Substance Use Disorder residential facility or Public or Private Child Service Entity is not utilized to replace or relocate a youth's primary residence. Room and board in Community Residential Group Home or Substance Use Disorder residential facility may be paid for by Medicaid for respite service only. In overnight settings of greater than 4 residents, between the hours of 7 am to 8 pm there shall be a minimum of one staff person on site for every five youth. Between the hours of 8 pm to 7 am there shall be a minimum of one staff person for every seven youth. At least one staff member must provide 24-hour awake supervision. The term "24 hour awake supervision" means that, during sleeping hours, a staff member is located in a position which allows him or her to observe any movement into or out of a youth's bedroom. On call staff must be available for emergencies and immediately accessible. The director or a designee shall be available at all times by cell phone.

ADDITIONAL REQUIREMENTS

Training

Providers are responsible for ensuring that all staff are appropriately trained to provide waiver services. Providers are responsible for the development and provision of training to their staff when alternative training is not available. Training records must indicate the name of the training course, the instructor's name, the training agency, the date of training, and the signature sheet or signed attestation for those in attendance. (Signatures must be legible.)

The following training must be administered to all staff who are responsible for delivering waiver services:

- **Pre-service Orientation:** Each staff member must sign an attestation indicating that they have been trained on the PCSC Waiver policy and document, understand the youth's person-centered plan of care, and the youth's unique or specific disability needs within the first 30 days of their employment. Staff should not have unsupervised contact with waiver participants until this training is completed.
- **Refresher Training:** This course is offered by SCDHHS and covers the waiver and policy and must be offered on an annual basis and/or in a time frame as dictated by CMS. Wraparound facilitators employed by COC at the time of waiver implementation will need to be trained within six months while those hired post-implementation must complete this training prior to seeing individuals. Signed attestations or training sign-in sheets must be kept as documentation of staff participation.
- **Job Specific Training:** COC will review the roles and responsibilities of a staff person's position prior to unsupervised contact with waiver participants. See the Service-Specific Qualifications of this section for information regarding job specific training requirements.
- **Other Required Training:** If there have been any substantial changes made to the waiver by SCDHHS or CMS, additional training will be required, as needed. The COC waiver point of contact or their designee will be notified if additional training is needed. Staff members are required to complete additional training only in their respective role(s) if the changes or quality issues impact their specific job function(s). Documentation should indicate that the information

presented in the required trainings was shared with those staff members who provide waiver services.

- Abuse and Neglect Training: This course will be offered by SCDHHS virtually or via material handouts and focuses on mandated reporter requirements, critical incidents, and requirements for reporting abuse/neglect and critical incidents.

Note: These requirements apply to all staff. Clinicians providing these services must meet requirements to maintain their licensure. Please see the Covered Services and Definitions section of this manual for more information about training requirements.

The following training must be completed by all staff who are responsible for administering CASII Assessments:

- CASII Training: This course teaches qualified bachelor's level and Licensed Practitioners of the Healing Arts, master's level clinicians, and intake workers how to conduct the CASII Assessment and determine the level of care needed by waiver participants. CASII assessors must complete training as verified by SCDHHS through training rosters before administering the CASII assessment to waiver participants.

Screening

The following screenings for all staff must be completed by the providers. These records must be completed upon the start of employment, and prior to unsupervised contact with the waiver participants. In addition, the following records must be verified, annually. This includes staff, volunteers, interns, and other individuals who are under the authority of providers who engage with PCSC Waiver participants.

- Criminal Record Check
 - Must be from an appropriate law enforcement agency
- Verification Child Abuse Registry Verification
 - Results must indicate no findings against an individual
- State and National Sex Offender Registries and the Child Abuse Registry
 - Results must indicate no findings or criminal charges against an individual

Documentation

Providers must have a file for each staff member, volunteer, intern, and other individuals who are under the authority of the provider, and who engage with PCSC waiver participants. This file must include sufficient documentation of all training, screening, and credentialing requirements, as it pertains to the individual's job. Providers must comply with all other applicable state and federal

requirements. Specific training requirements for each service type can be found in the section describing that service.

Limited English Proficiency

SCDHHS assures that it provides meaningful access to waiver services by Limited English Proficient persons in accordance with: (a) Presidential Executive Order 13166 of August 11, 2000 (65 FR 50121) and (b) Department of Health and Human Services "Guidance to Federal Financial Assistance Recipients Regarding Title VI Prohibition Against National Origin Discrimination Affecting Limited English Proficient Persons" (68 FR 47311 - August 8, 2003).

The waiver intake process ensures that the language needs of participants are assessed; and when appropriate, interpreter services will be provided. All waiver service providers must have a protocol system in place that allows access to services for persons with limited English proficiency. Designated staff, within the provider agencies, will be responsible for assuring compliance and access to services for persons with limited English proficiency.

The providers will be responsible for notifying SCDHHS of any discrimination complaints that are filed due to limited English proficiency and for submitting complaints to the Office of the Deputy Children's Advocate. Providers are responsible for maintaining records, documenting any filed complaints, resolving complaints, and documenting complaint resolutions. These records will be reviewed by SCDHHS, as part of the overall quality assurance process.

The provider agency can request assistance from SCDHHS, who will assist them in identifying resources, when/if necessary. SCDHHS requires that each provider agency be in compliance with Title VI of the Civil Rights Act of 1964 and CFR title 45 part 80. Providers must establish a grievance procedure to assure that everyone is given a fair and timely review of all complaints that allege discrimination. Prior to enrollment approval, all providers must agree, in writing, to the "Assurance of Compliance" statement. The SCDHHS Civil Rights Division must be notified of any complaints.

Emergency Safety Intervention (ESI)

The Emergency Safety Intervention (ESI) policy applies to any community-based provider that has policies prohibiting the use of seclusion and restraint, but who may have an emergency situation requiring staff intervention. Providers must have a written policy and procedure for emergency situations and must ensure that direct care staff are prepared and trained in the event of an emergency. If the provider intends to use restraint and/or seclusion, the provider is responsible for adhering to the following requirements:

- Providers must ensure that all staff involved in the direct care of a beneficiary successfully complete a training program from a certified trainer in the use of restraints and seclusion. This must be accomplished prior to ordering or participating in any form of restraint.
- Training should be aimed at minimizing the use of such measures, as well as ensuring the beneficiary's safety. For more information on selecting training models, visit <http://www.frcdsn.org/rest.html> to view the Project Rest Manual of Recommended Practice.

- Providers must have a comprehensive written policy that governs the circumstances in which seclusion and restraint are being used. This policy must adhere to all state licensing laws and regulations (including all reporting requirements).

Failure to have these policies and staff trainings in place, at the time services are rendered, will result in termination from the Medicaid program, and possible recovery of payments.

Critical Incidents

The State does not permit or prohibit the use of seclusion. Please see below the guidelines as specified by each State Agency:

1. Department of Social Services (DSS) - conducts yearly licensure reviews.
2. DSS; Out of Home Abuse and Neglect Division (OHAN) - investigates reports of any abuse or neglect by licensed facilities. Such reports can be made anonymously.
3. Referring State Agencies – All child serving state agencies are required to monitor and follow-up at least monthly, with each child they have placed in a licensed facility. Licensed facilities are required to file a Critical Incident (CI) report to the referring state agency within 24 hours from the time of the incident. CIs include any emergency safety intervention.

SCDHHS Quality Reviews are conducted annually or as needed. Reviews of respite provided in Therapeutic Foster Care homes will include a review of any use of seclusion reported to be used for children while participating in PCSC Waiver Respite.

4

COVERED SERVICES AND DEFINITIONS

COVERED SERVICES

Level of Care Evaluation

All waiver participants must be evaluated for Level of Care by a person who has successfully completed CASII training and meets the requirements of a Licensed Practitioner of the Healing Arts (LPHA) or a bachelor's level wraparound facilitator who is certified by SCDHHS as meeting high-fidelity wraparound standards.

Level of care is established at initial referral and annually thereafter.

Initial evaluation

The Child and Adolescent Service Intensity Instrument (CASII) is used to establish the participant's Level of Care.

The CASII is completed during the eligibility determination phase after a referral is received via an interview with the youth (and parent(s) when possible) and additional supporting information. The CASII takes into consideration child development and the importance of the parents and the community in supporting the child. It is used to determine the intensity of needed services. A youth is considered to meet the hospital facility level of care if the youth would be in need of a hospital placement but for the continued receipt of waiver services.

The CASII has six dimensions: Risk of Harm, Functional Status, Co-Morbidity, Recovery Environment, Resiliency and Treatment History, and Acceptance and Engagement. Each dimension has a five-point rating scale. For each of the five possible ratings within each dimension, a set of criteria is clearly defined. Only one criterion needs to be met for that rating to be selected.

The CASII hospital level of care determination is set at the same levels as the South Carolina hospital level of care. This results in determinations that are comparable to the SCDHHS hospital certification level of care criteria.

Re-evaluation of Level of Care

In addition to input from the youth and family, the evaluator will consider information gathered from supporting documentation in the youth's record. At least every 12 months, or more frequently if necessary due to a significant change in the youth's condition or needs, a youth's level of care must be reevaluated for continued participation in the program.

A youth is considered to meet the hospital facility level of care if the youth would be in need of a hospital placement but for the continued receipt of waiver services. The reevaluation also takes into account clinical evidence of therapeutic clinical goals that must be met before the individual can transition to a less intensive level of care and clinical evidence of symptom improvement.

SCDHHS and the waiver youth's wraparound facilitator both monitor the annual re-evaluation date through an electronic system to ensure that the level of care is completed every twelve months or if there is a significant change in the youth's condition or needs.

COC will conduct and score all LOC evaluations. All LOC scores and final LOC determinations will be entered and stored in the Phoenix system.

Person-Centered Plan of Care

The person-centered plan is the map for the family to ensure that there are sufficient supports and services for the youth to be supported in the home. The person-centered plan will be located in the Phoenix system and COC will manage access and plan maintenance. The person-centered POC must include:

- Identifying information
- Strengths and support needs for the waiver youth
- Clear language in the clinical justification section to ensure understanding of why the youth is receiving waiver services
- The provider of the services
- Type of services
- Frequency of services
- Duration of the services
- Goals for each service type that the youth, their family and the team have identified through the person-centered plan development process. Goals are stated from the perspective of the youth to reinforce the person-centered emphasis on waiver supports and services.
- The positive interventions and supports used prior to any modifications to the person-centered plan
- Documentation of less intrusive methods of meeting the need that have been tried but did not work
- A clear description of the condition that is directly proportionate to the specific assessed need
- A collection and review of data to measure the ongoing effectiveness of the modification
- Established time limits for periodic reviews to determine if the modification is still necessary or can be terminated (e.g., every three months or 90 days).

- Assurance that interventions and supports will cause no harm to the youth, located in the Rights and Responsibilities statement that is signed by the individual and/or their representative
- Where appropriate, the person-centered plan details how the staff/clinician/provider supports the waiver youth to reach their goal
- The setting in which the youth resides as chosen by the youth (this is specific to individuals living in Group Homes and should be included in the narrative checklist in Phoenix)
- Clinical and support needs as identified through an assessment of functional need
- Risk factors and measures in place to minimize them, including individualized back up plans and strategies when needed
- The name of the youth and/or entity responsible for monitoring the plan
- The informed consent of the youth in writing, and signed by all youth and providers responsible for its implementation

Review of the person-centered plan

The person-centered plan must be reviewed at least every 3 months and revised upon reassessment of functional need when the youth's circumstances or needs change significantly, or at the request of the youth.

Once the person-centered plan is reviewed by SCDHHS within three business days, the wraparound facilitator must send a full copy of the person-centered plan to each of the service providers as well as a copy for the family and youth.

High Fidelity Wraparound Care Coordination

The function of performing wraparound facilitation is to identify who should be involved in producing a community based, person-centered plan to meet the needs of the participating youth. The youth's family, extended family and other community members comprise the child and family team and play a vital role in the development of the person-centered plan.

Wraparound facilitation forms the child and family team which consists of the youth's family, extended family, and other community members involved in the youth's daily life for the purpose of producing a community-based, person-centered plan. This includes working with the family to identify who should be involved in the child and family team and assembly of the child and family team for the person-centered plan development meeting.

The child and family team is a multi-disciplinary team trained in or acclimated to High Fidelity Wraparound. Decisions on the plan of care are supervised by credentialed individuals meeting the requirements of a Licensed Practitioner of the Healing Arts (LPHA includes a psychiatrist, psychologist, LMSW, LPC, LISW-CP, LISW-AP, or LMFT) or individual employed by a public entity.

In addition to the youth and their family, wraparound teams include:

- Wraparound Facilitator
- Wraparound Team Leads
- Wraparound Supervisor/Coach

Wraparound Facilitator

The wraparound facilitator is responsible for the development of the person-centered plan. Together, the child and family team performs the three functions of HCBS care management: person-centered planning, referral to services, and monitoring of health and welfare and service delivery. Services provided to youth must include communication and coordination with the family and/or legal guardian.

During wraparound facilitation and plan reviews, coordination with other child serving systems should occur as needed to achieve the treatment goals. The wraparound facilitator guides the person-centered plan development process of the team to assure that waiver rules are followed. The wraparound facilitator also is responsible for reassembling the team when subsequent person-centered plan review and revision are needed, at minimum every 90 days to review the person-centered plan and more frequently when changes in the youth's circumstances warrant changes in the person-centered plan.

The wraparound facilitator emphasizes building collaboration and ongoing coordination among the family, caretakers, service providers, and other formal and informal community resources identified by the family. The wraparound process promotes flexibility to ensure that appropriate and effective service delivery to the youth and family/caregivers. More specifically, the high-fidelity wraparound process accommodates the needs of the participant and family in the provision of services.

The wraparound facilitator is to ensure the youth's involvement in decisions that affect his/her care, daily schedules and lifestyle. The overall atmosphere of the child and family team meeting setting is conducive to the achievement of optimal independence, safety and development by the youth with his/her input. The service is designed to be delivered in community settings including, but not exclusively in the youth's home.

Home and Community-Based Settings Validation

According to Appendix C of the approved waiver, through regular health and welfare monitoring, wrap facilitators observe the residence of all participants to ensure settings meet established home and community-based settings standards found at 42 CFR 441.301(c)(4) at enrollment annually or when the participant's residence changes. The wraparound facilitators shall document home and community-based settings observations in the Phoenix system. SCDHHS monitors, reviews and validates compliance with the setting requirements and enforces compliance actions, as necessary.

Verification occurs in coordination with regular health and welfare monitoring performed by wrap facilitators.

Home and community-based settings must have all the following qualities based on the needs of the youth as indicated in their person-centered plan:

- The setting is integrated in and supports full access of youth to the greater community, including opportunities to seek employment and work in competitive integrated settings, engage in community life, control personal resources, and receive services in the community, to the same degree of access as youth not receiving waiver services.
- The setting is selected by the youth or his/her guardian from among setting options including non-disability specific settings and an option for a private unit in a residential setting. The setting options are identified and documented in the person-centered plan and are based on the youth's or guardian's needs, preferences, and, for residential settings, resources available for room and board.
- The setting ensures a youth's rights of privacy, dignity and respect, and freedom from coercion and restraint.
- The setting optimizes, but does not regiment, age-appropriate youth initiative, autonomy, and independence in making life choices, including but not limited to, daily activities, physical environment, and with whom to interact.
- The setting facilitates youth and guardian choice regarding services and supports, and who provides them.
- In a provider-owned or controlled residential setting, the following additional conditions must be met:
 - The unit or dwelling is a specific physical place that can be owned, rented, or occupied under a legally enforceable agreement by the guardian of the youth receiving services, and the guardian of the youth has, at a minimum, the same responsibilities and protections from eviction that tenants have under the landlord/tenant law of the State, county, city, or other designated entity. For settings in which landlord tenant laws do not apply, the State must ensure that a lease, residency agreement or other form of written agreement is in place for each HCBS participant, and that the document provides protections that address eviction processes and appeals comparable to those provided under the jurisdiction's landlord tenant law.
- Each youth has age-appropriate privacy in their sleeping or living unit:
 - Units have entrance doors lockable by the youth, with only appropriate staff having keys to doors.
 - Youth sharing units have a choice of roommates in that setting.

- Youth have age-appropriate freedom to furnish and decorate their sleeping or living units within the lease or other agreement.
- Individuals have age-appropriate freedom and support to control their own schedules and activities and have access to food at any time.
- Individuals are able to have visitors of their choosing at any age-appropriate time.
- The setting is physically accessible to the youth.
- Any modification of the additional conditions must be supported by a specific assessed need and justified in the person-centered plan. The following requirements must be documented in the person-centered plan:
 - Identify a specific and individualized assessed need.
 - Document the positive interventions and supports used prior to any modifications to the person-centered plan.
 - Document less intrusive methods of meeting the need that have been tried but did not work.
 - Include a clear description of the condition that is directly proportionate to the specific assessed need.
 - Include regular collection and review of data to measure the ongoing effectiveness of the modification.
 - Include established time limits for periodic reviews to determine if the modification is still necessary or can be terminated.
 - Include the informed consent of the guardian of the youth.
 - Include an assurance that interventions and supports will cause no harm to the youth.
- Home and community-based settings do not include the following:
 - A nursing facility;
 - An institution for mental diseases;
 - An intermediate care facility for youth with intellectual disabilities;
 - A hospital; or
 - Any other locations that have qualities of an institutional setting, as determined by the Secretary.

Any setting that is located in a building that is also a publicly or privately operated facility that provides inpatient institutional treatment, or in a building on the grounds of, or immediately adjacent to, a public institution, or any other setting that has the effect of isolating youth receiving Medicaid HCBS from the broader community of youth not receiving Medicaid HCBS is presumed to be a setting that has the qualities of an institution unless the Secretary determines through heightened scrutiny, based on information presented by the State or other parties, that the setting does not

have the qualities of an institution and that the setting does have the qualities of home and community-based settings.

All coordination must be documented in the youth's waiver participant record. Child and family teams must receive ongoing and regular clinical supervision by a person meeting the qualifications of a Licensed Practitioner of the Healing Arts with experience regarding this specialized mental health service, and such shall be available during CFT meetings to provide back up, support, and/or consultation.

Service Limitations

Wraparound facilitation may not be provided at the same time as targeted case management and does not duplicate any Medicaid State Plan Service or services otherwise available to the recipient at no cost.

In compliance with HCBS requirements, individuals do not reside or receive services in any of the following settings:

- Any setting that is located in a building that is also a publicly or privately operated facility that provides inpatient institutional treatment,
- Any setting that is located in a building on the grounds of, or immediately adjacent to, a public institution,
- Any other setting that has the effect of isolating youth receiving Medicaid HCBS from the broader community of youth not receiving Medicaid HCBS

Service Exceptions

In the event the need for an out of home placement arises for a participant receiving HCBS waiver services, the state certifies all residences will be observed by the wrap facilitator to ensure settings are in the community and meet the standards at 42 CFR 441.301 (c)(4) as well as (vi)(A through F) for provider-owned and controlled settings, at placement and on a monthly basis through regular health and welfare monitoring.

In the case of Foster Care and Therapeutic Foster Care placements, the state certifies all homes are individually licensed by South Carolina Department of Social Services as foster care homes per S.C. Code Ann. 63-11-10 thru 63-11-790 (Supp 2008). The state will verify that all foster care homes annually meet licensing regulations as established and certified by South Carolina Department of Social Services and meet the standards at 42 CFR 441.301 (c)(4) as well as (vi)(A through F) for provider-owned and controlled settings, at placement and on a monthly basis through regular health and welfare monitoring.

In the event the need for Group Home placement for a waiver participant arises, the state certifies all residences will be observed by the wrap facilitator to ensure settings are in the community and meet the standards at 42 CFR 441.301 (c)(4) as well as (vi)(A through F) for provider-owned and

controlled settings, at the initial placement and on a monthly basis through regular health and welfare monitoring. The state will also verify Group Homes meet licensing standards as set forth by SC Code 63-11-30: <https://www.scstatehouse.gov/code/t63c011.php> and SC Regulation 114-590: <https://www.scstatehouse.gov/coderegs/Chapter%20114.pdf>.

Respite

Respite care includes services provided to beneficiaries unable to care for themselves furnished on a short-term basis because of the absence or need for relief of those persons who normally provide care for the beneficiary. Respite may be provided in an emergency to prevent hospitalization.

Respite provides planned or emergency short-term relief to a beneficiary's unpaid caregiver or principle caregiver who is unavailable to provide support. This service will be provided to meet the beneficiary's needs as determined by an assessment performed in accordance with SCDHHS requirements and as outlined in the beneficiary's person-centered plan. Beneficiaries are encouraged to receive respite in the most integrated and cost-effective settings appropriate to meet their respite needs.

Respite services may include camp funding for summer camp that provides the needed safeguarding services and supports to achieve the person's valued outcomes, as per the person-centered plan. Camps can either be focused on supporting youth with disabilities or camps that are available to the general public.

Services must be delivered in a manner that supports the beneficiary's communication needs including, but not limited to, age appropriate communication, translation services for beneficiaries that are of limited-English proficiency or who have other communication needs requiring translation, assistance with the provider's understanding, and use of communication devices used by the beneficiary. If the beneficiary is to receive respite on an ongoing basis, the care manager will monitor on a quarterly basis. The respite service may also be used to offer relief to a beneficiary's unpaid or principle caregiver who normally provides care.

Respite may be provided in an emergency to prevent hospitalization.

If the beneficiary is to receive respite on an ongoing basis, the Wrap Facilitator will monitor on a quarterly basis, as applicable, to see if the objectives and outcomes are being met.

Service Limitations

Respite services may not be longer than one week per episode and in aggregate, may not exceed 21 days per year. Stays of greater than 72 hours require prior authorization. Youth requiring crisis respite for longer periods may be evaluated on an individual basis and approved for greater length of stay based on medical necessity. In settings where permissible and during respite hours in the youth's home, medication may be administered by the respite worker. Medications, including controlled substances, medical supplies, and those items necessary for first aid shall be properly managed in accordance with State, Federal, and local laws and regulations. Such management shall address securing, storing, and administering medications, medical supplies, first aid supplies,

and biologicals, their disposal when discontinued or expired, and their disposition at discharge of a participant.

Respite is a face-to-face service.

Respite service may be offered in a foster care home, provided the foster home meets all qualifications and is enrolled with SCDHHS as a respite provider.

Respite services may not be billed at the same time as personal care services.

Respite may not be provided by any person who is a legal guardian or acting in the place of a legal guardian for the youth. A person with informal responsibility for the care of the youth such as a neighbor, friend, or extended family member may register with any of the provider types listed below. They must meet the provider's qualifications for employment, become an employee of the provider, and provide respite services for the youth. Respite may not be provided by any spouse or domestic partner of any person who is a legal guardian or acting in the place of a legal guardian for the youth.

In certified and licensed settings where permissible and during respite hours in the youth's home, medication may be administered by the respite worker in the youth's home. Medications, including controlled substances, medical supplies, and those items necessary for the rendering of first aid shall be properly managed in accordance with State, Federal, and local laws and regulations. Such management shall address the securing, storing, and administering of medications, medical supplies, first aid supplies, and biologicals, their disposal when discontinued or expired, and their disposition at discharge of a participant.

SCDHHS or its designee may authorize service request expectations above these limits on a case-by-case basis when it determines that:

- No other service options are available to the beneficiary, including services provided through an informal support network; and
- The absence of the service would present a significant health and welfare risk to the youth.

Respite service provided in a certified residential care facility, Substance Use Disorder residential facility or Public or Private Child Service Entity is not utilized to replace or relocate a youth's primary residence. Room and board in Community Residential Group Home or Substance Use Disorder residential facility may be paid for by Medicaid for respite service only. In overnight settings of greater than 4 residents, between the hours of 7 am to 8 pm there shall be a minimum of one staff person on site for every five youth. Between the hours of 8 pm to 7 am there shall be a minimum of one staff person for every seven youth. At least one staff member must provide 24-hour awake supervision. The term "24-hour awake supervision" means that, during sleeping hours, a staff member is located in a position which allows him or her to observe any movement into or out of a

youth's bedroom. On call staff must be available for emergencies and immediately accessible. The director or a designee shall be available at all times by cell phone.

Overnight settings offer a supportive home-like environment with a maximum preferred capacity of not to exceed 10 residents (fewer in rural areas), preferably in single rooms.

- The setting must be building and health and safety code compliant.
- Staffed and open 24 hours a day, seven days a week when a resident is present.
- Residents should be allowed to leave and return as needed, maintaining employment and other daily activities to the extent possible.

To the greatest extent possible, residents are encouraged to maintain contact with significant others, including family members, friends, and spouses. To facilitate this contact, residents may have visitors at any time that is convenient and practical for the resident as well as that of the overnight setting.

Exclusions

- Diagnosis of dementia, organic brain disorder or traumatic brain injury
- Displays symptoms indicative of active engagement in substance use manifested in aggressive or destructive behavior
- Respite service being used to replace or relocate an individual's primary residence

Medical Necessity Criteria

- The service is recommended by the wraparound facilitator and the youth in collaboration, and the service is included in the youth's person-centered plan; and
- There are emotional and/or behavioral problems which stress the ability of the caregiver or youth to provide for the youth's needs in the home thus putting the youth at risk of requiring a more intensive level of care (e.g., strained family relationships; exhaustion in caregiver; caregiver struggles to meet other work/family responsibilities or lacks enough time to care for self needs; increased symptoms of mental illness or substance use, such as psychotic thinking, sleeplessness, or self-injurious behavior) or the primary caregiver has a time limited situation that necessitates assistance in providing care for the youth (e.g., caregiver is experiencing an acute medical problem, caregiver must attend to a family crisis); and
- The absence of the service would present a significant health and welfare risk to the youth (e.g., youth has a seizure disorder, youth needs assistance with medication administration, youth has difficulty appropriately regulating water intake)
- No other means of temporary care exists; and

- The frequency and intensity of the service aligns with the unique situation of the youth and/or caregiver. Examples include:
 - A caregiver requests respite for a particular weekend so he can have a break and visit family members out of state.
 - A youth has a goal on her person-centered plan to appropriately limit her water intake, since it has led to electrolyte imbalances numerous times in the past, and agrees that she continues to need support in order to achieve that goal. As a result, she and her sister (who she lives with) have identified the need for pre-planned respite services one afternoon each month so the sister can go to a movie or shopping alone.
 - A caregiver has identified the immediate need for respite services for up to a week because the youth has been awake most of the night for the last three nights, resulting in the caregiver being awake as well and unable to go to work. The caregiver believes the youth's mood and sleep cycle will normalize in about a week, as it has done many times before, but respite allows the youth to avoid a higher level of care and provides a needed break for the caregiver.

5

UTILIZATION MANAGEMENT

PRIOR AUTHORIZATION

Respite Services

The participant's person-centered service plan must include a request for respite services, to include number of units requested. Prior authorization includes review from DHHS to ensure records include:

- Appropriate request for respite in the person-centered service plan
- Appropriate level of care designation

SCDHHS prior authorization alerts will be sent via electronic notification to Wrap Facilitators who generate a list of qualified respite providers for the participant and their team.

Freedom of Choice

Participants select and rank three (when available) respite providers in order of preference which the Wrap Facilitator submits as a Phoenix system referral. Respite providers can accept, decline or ignore a referral. Once a referral is accepted, Wrap Facilitators may provide additional information to the accepted provider after the referral has been accepted.

All changes to the units or services in the person-centered service plan must be resubmitted for prior authorization or risk not being reimbursed. SCDHHS will audit requests every quarter to ensure that person-centered plans of care and Level of Care designation meet compliance standards.

6

REPORTING/DOCUMENTATION

COST REPORTS

COC is required to submit an original of the prescribed SCDHHS cost report to include actual, allowable, necessary and reasonable cost and service delivery information for each service reflected in Appendix A of the service contract. The report must be completed and mailed to SCDHHS within one hundred twenty (120) days after the end of COC's fiscal year if the Contract is written for a period greater than one year. Failure to file any given report within specified time frames will result in all future reimbursements being withheld until report is received.

All cost reports should be mailed to:

Department of Health and Human Services
Division of Ancillary Reimbursements
Post Office Box 8206
Columbia, SC 29202-8206

If you have questions regarding cost reports, contact the SCDHHS Medicaid Provider Service Center at 1-888-289-0709 or submit an online inquiry at <http://www.scdhhs.gov/contact-us>.

DOCUMENTATION REQUIREMENTS

General Requirements

COC and waiver providers are required to maintain documentation in the Phoenix system. Documentation is required to record any case activity in accordance with policy and procedures and is mandatory. Documentation provides an accurate record of all case activities and actions. All documentation is a legal record that may be used as evidence in court. Documentation is also required to ensure organizational and worker compliance. Phoenix Narrative checklists and Conversations are part of the narrative record.

All communication between COC Representatives/workers and participants, primary contacts, providers and other related agencies must be through verbal information exchange during face-to-face visits or by phone, mail or by fax. Electronic methods such as texting, emailing, tweeting, etc. are not permitted.

Phoenix System Requirements

All documentation in the participant's Phoenix system profile should:

- Be related to the participant and reflect factual information recorded in a manner consistent with professional and licensure standards
- Use only agency approved abbreviations

- Include correct spelling and grammar
- Exclude extraneous comments unrelated to the participant's situation
- Exclude personal opinions and judgmental statements
- Exclude emails
- Exclude quality assurance reviews/forms

Conversations Function

The Conversation Tab feature may be used rather than contacting the provider by telephone. However, when informing the provider of information that requires immediate attention such as a termination or change of facilitator, a telephone call for verbal communication is necessary. The first and last name and title, if applicable, of any person from whom information is received must be documented in the narrative. The primary contact and the physician are to be identified by first and last name in the narrative, as the individuals assuming these roles may change over time. All documentation policies apply to the Conversation Tab feature in Phoenix.

Documentation Timeliness Standards

All waiver documentation for the month must be keyed, entered and uploaded to the network by 11:59 pm of the last day of each month. This supersedes any documentation timeliness and policy guidelines. For example, documentation related to re-evaluations completed the last week of the month but not due until the first week of the following month, must be completed by 11:59 pm of the month the visit was completed.

When counting days for timeliness standard issues, the first day is determined to be the next business or calendar day depending on the standard. For example, the timeliness standard for documentation is three business days. For an activity that occurred on a business day Monday, Day One (1) would be Tuesday. Day Two (2) would be Wednesday. Day Three (3) would be Thursday. Therefore, the activity must be documented by 11:59 pm on Day Three (3), Thursday. For an activity that occurred on a business day Friday, Day One (1) would be Monday. Day Two (2) would be Tuesday. Day Three (3) would be Wednesday. Therefore, the activity must be documented by 11:59 pm on Day Three (3), Wednesday. However, all documentation for the month must be keyed, entered and uploaded to the network by 11:59 pm of the last day of each month regardless of timeliness standards.

Supporting Document Standards

A fax copy of a document may be accepted and suffice for documentation in the record if legible. The original must be scanned into Phoenix Scans. If a document is faxed to someone, there is no need to follow-up with an original by mail. Routing methods such as mail or fax should be used when Phoenix is not an option.

All documents that are not available in Phoenix are to be scanned to the participant's Phoenix file. All scanned documents are to be stored under the appropriate scan tag in Phoenix Scans. Refer to the "Scan Tag Descriptions" document in Phoenix HELP to determine the appropriate scan tag for document placement.

All documents received from outside sources, are to be scanned in the participant's Phoenix file by 11:59 pm on the last day of each month unless the document is received within the last three business days of the month. In this situation, the document should be scanned in the participant's Phoenix file by the 5th day of the following month.

Packets of supporting medical records and documentation received in conjunction with a level of care request may be scanned as one packet in Phoenix scans under the appropriate scan tag. The Consent Form and Participant Service Choice Form will be separated from the packet and scanned under the designated scan tags.

If hard copy Consent Forms or Participant Service Choice Forms are obtained for any reason, these forms must be scanned into the appropriate tag in Phoenix scans within three business days.

For the enrolled waiver participant, the hard copy form will be retained until the next quarterly visit at which time the scanned document is confirmed to be present and readable in Phoenix Scans under the appropriate scan tag. For all other types of applications, the hard copy form will be disposed of when the scanned document is confirmed to be present and readable in Phoenix Scans under the appropriate scan tag.

All providers must follow their agency's HIPAA record disposal policy.

Complaints

When reporting a complaint in Phoenix, the report must include, if appropriate, the name of provider, name of provider worker, and provider worker ID.

Participant problems or needs should be documented diligently and include follow up documentation, as appropriate. If a need is identified in the narrative, any subsequent actions taken on the participant's behalf must be included in the narrative. All contacts made with the participant or on the participant's behalf must be narrated. All contacts and actions must be narrated within three (3) business days of the activity, unless a shorter period of time is otherwise specified in policy and procedures, contracts, or other official SCDHHS communications.

The date noted in the "Date of Contact" field in the narrative must be the date the activity or contact was completed. The date of the contact may also be entered in the text of the narrative. Narrative documentation indicating activities completed over several days are not appropriate.

Documentation Retention Policy

Purpose

The purpose of this policy is to provide a system for complying with document retention laws to ensure that necessary records and documents are adequately protected and maintained as well as to ensure that records that are no longer needed or are of no value are discarded appropriately. This policy is intended to assist COC in the retention of any hard copy files located onsite. This policy is applicable to records of any applicant or participant in the PCSC waiver.

Policy Details

The South Carolina Department of Health and Human Services (SCDHHS) has the following requirements related to record retention. Other state or federal rules may require longer retention periods. "For Medicaid purposes all fiscal and medical records shall be retained for a period of five (5) years after the last payment was made for services rendered, except that hospitals and nursing homes are required to retain such records for six (6) years after the last payment was made for services rendered." Additionally, "if litigation, claim, audit, or other action involving the records has been initiated prior to the expiration of the appropriate retention period, the records shall be retained until completion of the action and resolution of all issues which arise from it or until the end of the appropriate retention period, whichever is later."

The PCSC waiver maintains compliance with this policy by maintaining participants' records in our case management system known as Phoenix.

Maintenance of Hard Copy Records/Documentation

In an effort to go paperless, any documents/records that must be maintained should be scanned into Phoenix using the appropriate scan tags. The hard copy documentation should be shredded. If an appropriate scan tag is not available, scan the document under "Other forms/Documents."

Note: Staff members should verify that a legible copy of the document is in Phoenix prior to the shredding of the documentation.

Documents that May be Shredded

- Any document in which the retention period has expired and there is no litigation, claim, audit, or other action involving the record.
- Any document within the retention period and the staff member can verify that a legible copy of the documentation is located in Phoenix. Some examples include but are not limited to:
 - Authorizations o Provider Choice Forms
 - PCSC Notification Forms
 - Consent and Rights and Responsibilities Forms
 - PCSC or COC Assessments
 - PCSC Service Plans
 - Certification Letters

Note: Additional guidance can be found on pages 22 – 28 of the Records/Documentation Requirements section of the SCDHHS Provider Administrative and Billing Manual.

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DISCHARGE

REASONS FOR DISCHARGE

Waiver participants will be discharged from the waiver when it is determined that they no longer meet one of the following requirements:

- Have a Severe Emotional Disturbance (SED) or Serious Mental Illness (SMI) and SUD.
- Are between ages 0 and 21
- No longer have a need for waiver services
- No longer meet Hospital Level of Care eligibility determination requirements
- Financial eligibility for Medicaid services
- Greater level of care needs than waiver services can provide

DISCHARGE PROTOCOL

Service Plan Development (SPD) teams must develop a transition plan for waiver participants, prior to a participant being discharged from waiver services. SCDHHS will notify waiver participants of termination of their waiver services once the required discharge documentation is received.

Discharge documentation must be submitted by COC to PCSC Waiver staff within 48 hours of the waiver participant's discharge from the waiver. SCDHHS will notify families via a termination form.

Service Specifics

During the SPD process, waiver participants may be discharged from a specific service but remain in the waiver. If providers feel that a waiver participant is no longer appropriately eligible for the services they deliver, the providers can recommend discontinuation of that service to the SPD team, for further discussion and consideration. Waiver participants are notified when there is a change in the Plan of Care. A right to appeal notification will be sent out by COC with a change of service notification.

Providers must discharge a waiver participant through the SPD team process. The only exception is when there is a health and/or safety risk for the waiver participant. In these instances, providers can suspend provisions for waiver services until the safety issues are addressed.

For Example: A waiver participant may become physically aggressive towards their respite staff. If the service program is not adequately staffed while the participant exhibits those behaviors, providers must immediately identify and communicate the risks to the participant's respective Wrap Facilitator. If this type of circumstance arises, the SPD team should determine how to best support

the waiver participant and/or their family, if the participant is discharged from the service as a result of the health and/or safety risks that they posed.

Health and Safety Needs

Upon review of the person-centered plan, if waiver services and/or providers appear incapable of assuring the safety and health of the waiver participant and those within the community SCDHHS reserves the right to discharge the waiver participant from the PCSC waiver and recommend inpatient level of care until the waiver participant's level of care can be met by waiver services.

Example: Waiver services and providers may not be capable of meeting CASII Level of Care 6 Recommendations, which would indicate that the waiver participant needs "secure, 24-hour management". The burden of proof is on the waiver provider to clearly indicate that waiver services can ensure the health and safety of the waiver participant. The burden of proof should be displayed and communicated via a Crisis Plan by COC, a formal letter justifying the waiver participant's continuation in the waiver, and any additional information requested by SCDHHS.

Interruption

Waiver participants who are hospitalized, or who are in a PRTF for 30 days or more, will be discharged from the waiver. If a participant chooses not to receive waiver services for 30 or more days, they will be discharged from the waiver. In the event of special circumstances, waiver participants may request that the 30-day timeframe be extended. Participants are required to provide an explanation, which identifies their need for an extension. COC communicates this information to SCDHHS through the critical incidents module, where a request for extension would be made.

There may be instances when significant risks to waiver participants are identified, and when SPD teams cannot adequately support the participant and/or their family through home- and community-based supports and services. In these situations, participants may be placed in a hospital or PRTF to ensure the health and safety of the participants and their families. SPD teams should work closely with waiver participants and their families and should anticipate the likelihood of potential crises among the participants. Ideally, the teams should have proactive supports and measures in place, in an effort to avoid the need for potential hospital or PRTF admissions. If a crisis presents serious health and/or safety risks that cannot be avoided, and hospitalization or PRTF placement is needed, participants may be admitted to a facility for up to 29 days without affecting their waiver eligibility. If a participant is admitted to a hospital or PRTF for 30 days or longer, they must be discharged from the waiver. Participants who were enrolled in the waiver service program within the past year, and who are transitioning out of a hospital or PRTF, can re-enroll in the waiver by receiving a crisis slot. However, those participants who have been discharged from the waiver for more than one year must re-apply for services.

Waiver participants who are admitted to a hospital or PRTF, or those who temporarily choose to interrupt their services for less than 29 days, are not required to be discharged. Upon reinstating waiver services, the person-centered plan of care does not need to be reviewed, unless the services are interrupted after the plan of care end date. If a plan of care has expired, participants may return

to waiver services based on the SPD team's review of the plan of care. In order to ensure that required services are provided in a timely manner, desk reviews can be conducted by the wrap facilitators. This will allow waiver services to restart while SPD team meetings are scheduled. Person-centered plan desk reviews must be submitted to SCDHHS, as required; and services must also be authorized. SPD teams must review person-centered plans immediately following the participant's return to waiver services. This ensures that the waiver services plan remains valid and continues to meet the needs of participants and/or their families.

Increase in Functioning

Waiver participants, who no longer meet level of care requirements because of an increase in functioning, will be ensured that necessary supports are in place for them and their families. These supports will help participants maintain success following the termination of their waiver services. Regular conversations during SPD team meetings should address the benchmarks that participants must reach, which will eventually lead to the reduction and elimination of their need for waiver services. Teams should be proactive regarding participant discharge. When participants no longer meet level of care requirements, SPD teams should already know what supports are needed for a successful discharge from the waiver.

Freedom of Choice

If a participant and/or their family determine that they no longer want waiver services, they can discharge from the waiver, without prior notice. Whenever possible, SPD teams or case managers should initiate discussions about the participant's decisions, and they should determine what supports are needed upon the participant's discharge from the waiver. Alternative supports and services should be discussed, as requested, with the waiver participants and/or their families. Appropriate referrals should be made, as requested, by the families. Participants and/or their families who do not accept supports from the SPD teams or case managers, and who also express an interest in immediate discharge, may do so. The discharging youth and or family can reject post-waiver supports from the SPD. This response needs to be recorded with signatures from the family, whenever possible, and SPD. If families disengage prior to discharge, COC is responsible for documenting this in the participant's Phoenix file and documenting attempts at re-engaging family, as per policy.

Aging Out

All waiver participants who are enrolled in the waiver at the time of their 21st birthday will be discharged from the waiver, as they will no longer be eligible to receive services through the waiver. Wrap Facilitators are responsible for keeping track of those waiver participants who will approach age 21, in order to ensure that a transition plan is developed at least 90 days prior to a participant's 21st birthday. During the months prior to the waiver participant's 21st birthday, their family will be given information regarding the transition planning procedures. SCDHHS staff or its designee can provide the families with information about other State Plan services that might be available to waiver participants upon their discharge.

Three months prior to a participant's aging out of the waiver, the family's SPD team will meet to develop a transition plan. Wrap Facilitators will be responsible for coordinating the transition team meetings. SCDHHS, and the SPD teams will work with participants and/or their families to ensure that they are aware of, and have access to available services, which will support the participants upon their discharge from the waiver.

The transition plan of care must be signed by a Wrap Facilitator, as well as the waiver participant and/or their family. The signature(s) designate that they approve of the transition plan. After the meetings, case managers will make the appropriate referrals to ensure that the necessary supports and services are in place upon a participant's aging out of the waiver. The participant's transition plan of care shall specify which transitional services are pursued on their behalf and shall also contain evidence that appropriate referrals/coordination has been initiated.

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BILLING GUIDANCE

GENERAL BILLING GUIDANCE

Federal Financial Participation

Federal financial participation (FFP) may not be claimed for waiver services that are furnished prior to the development of the service plan or for waiver services that are not included in an individual's service plan. A service plan may not be backdated.

All billing is completed in accordance with SCDHHS billing guidelines and only if services meet billing criteria found at the link below. All services billed should include corresponding clinical documentation in the participant's Phoenix system file.

Billable Code/Location of Service

The following list provides the codes most commonly used:

- 03 — School
- 11 — Clinician or Doctor's Office
- 12 — Home
- 19 — Off Campus Hospital
- 22 — Outpatient Hospital
- 23 — Emergency Room
- 53 — Community Mental Health Center
- 55 — Substance Abuse Residential Facility
- 57 — Non-Residential Substance Abuse Facility
- 99 — Other Unlisted Facility (excluding recreational settings)

SERVICE-SPECIFIC BILLING GUIDANCE

Refer to the service information (linked in the Covered Services and Definition section above) associated with this manual for additional information on the service definitions for the services listed below.

Level of Care

A level of care evaluation is required to determine waiver eligibility. Staff employed by the SC Continuum of Care and trained in the administration of the Child and Adolescent Service Intensity Instrument (CASII) will conduct an initial and annual level of care assessment. Payment for level of care evaluations are included in the High Fidelity Wraparound monthly reimbursement rate.

Billable Place of Service

Level of care assessments can be conducted telephonically or face-to-face.

Person-Centered Plan of Care

Staff employed by the SC Continuum of Care along with the participant, family and child and family team are responsible for the development of the Person-Centered Plan of Care. A review occurs every 90 days. Payment for development and review of the person-centered plan of care is included in the High Fidelity Wraparound monthly reimbursement rate.

Billable Place of Service

Person-Centered Plans can be developed in several locations.

High Fidelity Wraparound Care Coordination

High Fidelity Wraparound is provided by staff employed by the SC Continuum of Care and is billed on a monthly basis.

Billable Place of Service

High Fidelity Wraparound must be provided in approved home and community-based settings identified in the Covered Services and Definitions section of this manual.

Respite

Respite is provided by providers in good standing and is billed directly to Medicaid.

Billable Place of Service

Respite must be provided in approved home and community-based settings identified in the Covered Services and Definitions section of this manual.

BILLABLE CODES

Billable codes can be found in the table below:

Service	Proc Code	Modifier	Limit	Unit
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Unskilled Respite Care, Center-based	H0045	U1	No longer than 1 week per episode, not to exceed a maximum of 21 days per year. Individual stays of greater than 72 hours require prior auth. Crisis respite for longer periods evaluated on individual basis and approved for greater length of stay based on medical necessity. Cannot be billed at same time as personal care services in state plan.	Per diem
Unskilled Respite Care, Non-center based, 15 minutes	S5150	U1	No longer than 1 week per episode, not to exceed a maximum of 21 days per year. Individual stays of greater than 72 hours require prior auth. Crisis respite for longer periods evaluated on individual basis and approved for greater length of stay based on medical necessity. Cannot be billed at same time as personal care services in state plan.	15 minutes
Unskilled Respite Care Services, Non-center Based Per Diem	S5151	U1	No longer than 1 week per episode, not to exceed a maximum of 21 days per year. Individual stays of greater than 72 hours require prior auth. Crisis respite for longer periods evaluated on individual basis and approved for greater length of stay based on medical necessity.	Per diem
Respite Care Services, Non-center Based 15 minutes	T1005	U1	No longer than 1 week per episode, not to exceed a maximum of 21 days per year. Individual stays of greater than 72 hours require prior auth. Crisis respite for longer periods evaluated on individual basis and approved for greater length of stay based on medical necessity. Cannot be billed at same time as personal care services in state plan.	15 minutes
High Fidelity Wraparound	T2022	U1	No limit, evidence-based practice. Cannot be billed on the same day as TCM/WCM.	Per Month